SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

FLOX: fluorouracil, folinic acid and oxaliplatin

Adjuvant stage II or III colon cancer after resection · NCCP 00486 · version 8 · 56-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
FLOX is given after surgery for selected stage II or III colon cancer to reduce the chance of recurrence. It combines weekly fluorouracil/folinic acid with oxaliplatin on alternate treatment weeks.

Schedule
Fluorouracil and folinic acid on Days 1, 8, 15, 22, 29 and 36; oxaliplatin also on Days 1, 15 and 29. Two weeks off follow. Three 56-day cycles make a planned 24-week course.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ DPD result and severe fluoropyrimidine risks reviewed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Oxaliplatin
PLATINUM CHEMOTHERAPY

- ☐ Cold-triggered tingling / throat discomfort
- ☐ Persistent sensory neuropathy, sometimes lasting after treatment
- ☐ Allergic / infusion reaction
- ☐ Low blood counts and infection
- ☐ Nausea, diarrhoea and fatigue
- ☐ Rare serious lung or blood complications

Folinic acid
CHEMOTHERAPY MODULATOR

- ☐ Enhances fluorouracil activity and toxicity
- ☐ Rare allergic reaction
- ☐ Diarrhoea / mouth soreness in combination

Fluorouracil
CHEMOTHERAPY

- ☐ Diarrhoea, dehydration and mouth ulcers
- ☐ Low blood counts / infection
- ☐ Hand-foot skin soreness
- ☐ Cardiotoxicity including chest pain / ischaemia
- ☐ Severe early toxicity including DPD-related toxicity

Confirm treatment aim, expected benefit, reasonable alternatives and no-treatment/supportive-care options. Discuss DPD, infection, mouth/bowel/skin toxicity, cardiac symptoms, fertility, contraception, breastfeeding and interactions. Discuss persistent neuropathy and infusion reactions.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00486 v8.

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